



Private and Confidential

11 June 2020

Mr Richard McLean
24 Rosella Court
OCEAN GROVE VIC 3226

By email: rich@richmclean.com.au

NHPOPC reference: OCF/20/50

Dear Mr McLean

Proposed outcome to your complaint

I am writing to you today regarding the complaint you made to the National Health Practitioner Ombudsman and Privacy Commissioner (NHPOPC) about the administrative actions of the Australian Health Practitioner Regulation Agency (Ahpra) and the Medical Board of Australia (the Board).

As previously advised in our letter to you dated 5 February 2020, the role of this office is to ensure that the administrative processes of Ahpra and the Board are fair and reasonable. We cannot change a decision made by Ahpra and the Board, but we can make recommendations or suggestions where we have concerns.

Background to your complaint

Your complaint concerns Ahpra and the Board's handling of the notification you made about Dr John Whitaker.

In summary you told us that:

- on 7 August 2018, you attended Dr Whitaker's rooms for a consultation and recorded your consultation with Dr Whitaker without his knowledge ("the recording")
- you informed Dr Whitaker that you were undergoing stress, depression, isolation and suicidal ideation
- Dr Whitaker did not address your concerns or offer you any support such as recommendation to see a psychologist or a psychiatrist
- Dr Whitaker further prescribed you a high dosage of Tramadol which you later misused in an attempt to commit suicide
- on 10 May 2019, you made a notification about Dr Whitaker to Ahpra and informed Ahpra of the recording
- Ahpra refused to accept the recording, or provide the recording to the Board for consideration
- you subsequently made a transcript of the recording ("the transcript") and provided it to Ahpra for the Board's consideration. Ahpra informed you that Dr Whitaker had the opportunity to "mark up" the transcript before it was presented to the Board
- on 8 November 2019, you were invited to attend a face-to-face meeting at Ahpra and you were provided with a letter informing you that the Board had decided to take no further action in relation to your notification.

NHPOPC complaint

On 26 January 2020, you complained to this office about Ahpra and the Board's handling of the notification you made about Dr Whitaker. You are concerned that the Board only considered Dr Whitaker's "marked up" version of the transcript, and that neither the recording nor the transcript was put to the Board before it made its determination.

Complaint issues considered

I identified the following complaint issues for investigation:

1. Whether it was reasonable for Ahpra to determine that the recording of 7 August 2018 should not be put to the Board for consideration?
2. Whether it was reasonable for Ahpra to provide the transcript to the Board for consideration?
3. Whether your notification was otherwise handled in a fair and reasonable manner?

Assessment of concerns and decision

As part of my investigation, I obtained the following information from Ahpra:

- a copy of your notification about Dr Whitaker
- all supplementary information provided by you in support of your notification
- Agenda papers considered by the Board in relation to your notification about Dr Whitaker and all supporting documents including the transcript, Dr Whitaker's response to the notification and the clinical records
- Decisions and Actions papers arising from the Board's considerations of your notification
- all written correspondence, including emails, that passed between Ahpra, yourself, Dr Whitaker and his legal representative in relation to the notification about Dr Whitaker
- records of telephone contact between Ahpra, yourself, Dr Whitaker and his legal representative, including file notes made by Ahpra about the subject matter of the call and any advice given in response.

In addition to the above, I have also considered the emails and supporting information that you have forwarded to our office.

After looking closely at all of the available information regarding your complaint, I am proposing not to investigate this matter further. I know this will be disappointing for you as it is not the outcome you were hoping for. I outline my reasons below and I hope that this helps you understand why I have reached this decision.

Issue 1: Was it reasonable that Ahpra did not put the recording to the Board for consideration?

You told us that on 7 August 2018, you recorded a consultation between Dr Whitaker and yourself, without Dr Whitaker's consent. I understand that you believe this recording should have been considered by Ahpra and the Board.

I have reviewed Ahpra's policy in relation to accepting recordings and I am satisfied that it acted in accordance with its policy. Ahpra's policy position is consistent with the law in Victoria which makes it unlawful to publish or communicate a record or report of a private conversation without the consent of all parties to that conversation.¹ While Ahpra acknowledges there are exceptions to this policy that allow

¹ *Surveillance Devices Act 1999 (Vic)*

recordings to be communicated to third parties, the Ahpra investigator handling your matter did not believe any exemptions would apply in your case.

Having considered all the available evidence surrounding the recording, I am satisfied that it was reasonable for the Ahpra investigator to determine that the recording should not be put to the Board for consideration in the absence of Dr Whitaker's consent. While I appreciate that this situation has been frustrating for you, Ahpra must ensure that it handles any information it receives in a lawful way.

Issue 2: Was it reasonable for Ahpra to provide the transcript to the Board for consideration?

A transcript may be considered a record or report of a private conversation under Ahpra's policy. This means communicating or publishing a transcript without the consent of all parties to that conversation may be an offence under the law in Victoria ². Ahpra and the Board will only accept a written statement of a private conversation in very limited circumstances which may include:

- where an individual's written recollection of a conversation is not a transcript of a private conversation or has not been generated from the use of a listening device
- where an individual provides a statement about a private conversation based on their recollection of the conversation and advises Ahpra and the Board that there is no reliance on their audio recording of that conversation.

During the course of my investigation, I sought clarification from Ahpra as to why it accepted your transcript and presented it to the Board for consideration. Ahpra's response was that the Ahpra investigator treated the transcript as a recounting of your memory of the consultation which meant that it was allowable under Ahpra's policy.

Having considered all of the available evidence in relation to this issue, I am satisfied that the transcript you provided to Ahpra represents a record of a private conversation. It appears there was reliance on the recording of the conversation between you and Dr Whitaker when you made the transcript. It is my view that Ahpra should not have accepted or provided a copy of your transcript to the Board without Dr Whitaker's consent. By doing so, I believe Ahpra was in breach of its policy.

I will ensure that this error is brought to Ahpra's attention. However, I do not believe this error requires that your notification should be reconsidered by the Board. In my view, such a reconsideration would be unlikely to make any difference to the outcome of your notification. The Board decided to take no further action against Dr Whitaker after considering the transcript which you believe contains evidence that is unfavourable to Dr Whitaker. If the Board was to reconsider your notification without the transcript, it would be presented with less information regarding your concerns about Dr Whitaker and it is therefore improbable that the Board would change its view that it is not required to take action against Dr Whitaker.

I have also considered whether the Board should reconsider the notification without the transcript to ensure the process is fair for Dr Whitaker. Procedural fairness dictates that it is only fair and just for a practitioner to be given the opportunity to respond to a notification, to allow presentation of both parties' account/interpretation/recollection of the consultation to the Board for its consideration. To this end, Dr Whitaker was given an opportunity to respond to your notification and the transcript. The Board was therefore presented with both your version of events (your notification materials) and Dr Whitaker's version of events (his submission in response to your notification). While it would have been better if Ahpra did not share the transcript with Dr Whitaker (because the transcript should not have formed part of the evidence presented to the Board), I consider that the information you and Dr Whitaker provided to Ahpra were given equal and accurate representation when presented to the Board. I am satisfied that the Board considered the issues that you raised in your notification, the transcript, and Dr Whitaker's

² *Ibid*

response. Having considered both points of view, the Board decided that no regulatory action was warranted.

In general, it is my view that Ahpra's handling of recordings and transcripts could be improved. Ahpra has acknowledged that better clarity should have been provided to its staff about the use of transcripts. Ahpra has given our office an undertaking that it will develop resources for its staff about the use of recordings and transcripts during the notifications process. We will continue to closely monitor Ahpra to ensure that there is consistency amongst all Ahpra staff in the handling of recordings and transcripts.

Issue 3: Was your notification otherwise handled in a fair and reasonable manner?

You believe that the recording proves that Dr Whitaker did not offer you any psychiatric help and prescribed a drug to you which provided you with a method of suicide. You also believe that Ahpra and the Board were biased in favour of Dr Whitaker.

After reviewing the available information, I am satisfied there is no evidence to support that Ahpra and the Board were biased. I am also satisfied that it was reasonably open to the Board to take no further action against Dr Whitaker. I explain below the reasoning for my decision.

Your concern that Dr Whitaker did not offer you any psychiatric help

From reviewing the supporting documents to your notification, I understand that you expressed suicidal ideation and asked Dr Whitaker for psychiatric help, but Dr Whitaker replied that he did not know anyone who could assist you. You state that the recording clearly demonstrates Dr Whitaker's refusal to offer you any help or provide you with a referral to another psychologist or psychiatrist.

The Board determined from your clinical records that you returned to Dr Whitaker for multiple consultations prior to your reported suicide attempt on 24 September 2018, some 6 weeks subsequent to the specified consultation. You attended further consultations with Dr Whitaker without complaint and given the time frame between the alleged concerns and your suicide attempt, the Board considered there to be lack of definitive correlation between these events.

I am satisfied that it was reasonably open to the Board to determine that Dr Whitaker's performance was not inappropriate, and that no further regulatory action was required. I consider that the clinical records which were presented to and considered by the Board were sufficient to demonstrate that Dr Whitaker had previously organised referrals for you.

Your concern that Dr Whitaker prescribed you Tramadol given your disclosed suicidal ideation

You further expressed concerns to Ahpra that given your disclosed suicidal ideation, Dr Whitaker's prescribing of Tramadol was negligent, in that it provided you with a method of suicide.

The Board, after reviewing your clinical records and Dr Whitaker's response, noted that you reported you were in pain from a longstanding shoulder injury. The Board considered that if this injury had been left untreated it could have further added to a psychological burden and increased a risk of suicide. As Tramadol was previously prescribed to you for a shoulder injury, the Board considered Dr Whitaker's prescribing of Tramadol appears in line with accepted professional standards of practice, being a safer drug than other moderately strong pain killers to prescribe in the described circumstance.

I am satisfied that it was reasonably open to the Board to determine that Dr Whitaker's performance was not inappropriate, and that no further regulatory action was required.

Conclusion

After giving careful consideration to your complaint, I am proposing not to investigate further. I am satisfied that it was reasonably open to the Board to conclude that Dr Whitaker's performance in this instance was not inappropriate, that he does not pose a risk to public health and safety and therefore no further regulatory action was required.

If you believe any of the information in this letter is incorrect or if you consider that I have misunderstood any part of your complaint, please provide me with your response by **25 June 2020**.

If I do not hear from you by the above date, I will assume that you do not wish to provide any comments and I will close your complaint.

I know your experience with Ahpra and the Board has been difficult for you, but I thank you for the time you have taken to contact our office and discuss your concerns with us. Every complaint helps us identify possible areas for improvement.

I have also warned you that in Victoria, it is illegal to publish a record of a telephone recording without consent from both parties. You are at risk of criminal sanctions for breaching the Act ³. For the protection of your lawful interests, I strongly urge you to remove the recording between yourself and our office that you have published on your website.

Yours sincerely

A handwritten signature in black ink, appearing to read 'Preya McKenzie', with a stylized, cursive script.

Preya McKenzie
Investigator

³ *Surveillance Devices Act 1999 (Vic)*, s.11(1)